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Essay 3 Rough Draft

The Systemic Failures Behind the American Mental Health Crisis

In June of 2015, President Barack Obama stood before the mourners of Charleston, South Carolina, and delivered a eulogy that grieved for much more than one life. He described communities that have grown passive, neighbors who have fallen silent, and a nation that has stopped reaching out to individuals suffering in silence. The grief shown by Obama was not only focused on the violence. It focused on the slow, invisible crisis that is unfolding beside it: the rising crisis of depression, mental illness, and suicide, which continues to be among the leading causes of death in the United States, especially for individuals between the ages of ten and thirty-five. For decades, the primary American response to mental illness was to place responsibility on the individual, labeling it as a lack of willpower, faith, or personal strength. The idea that suffering is a private burden to be managed quietly came from the cultural norms of society, religious institutions, and healthcare providers who treat mental illness as secondary to physical health. **It was not until the 2010s, as suicide rates began to climb, that the national conversation was forced: what if the system itself is producing the suffering? That question remains contested by critics who argue that overmedicalizing distress removes individual accountability and creates a culture of dependency. Others insist that the mental health crisis is simply a pharmaceutical problem – that medication alone is the clinically appropriate answer and that communal arguments distract from clinical solutions.**

However, a growing body of peer-reviewed research points in a different direction, revealing four key systemic failures that require no prescription to fix. The rising depression, mental health issues, and suicide in America are not the result of individual weakness, but are driven by communal failures – including the collapse of communal and social hubs, the stigma surrounding mental illness, the psychological harm of social media, and the unequal access to mental healthcare – all of which demand national action.

The collapse of communal spaces in America has created a gap that the healthcare system alone can not fill, leaving millions of struggling people with no safety net to catch them. Hadassah Buechner and colleagues, in their 2023 systemic review published in the Australian and New Zealand Journal of Psychiatry, found that “healthcare systems do not currently have the capacity to deliver interventions to treat or prevent mental illness to all who would potentially benefit” (Buechner et al. 643) The review used randomized controlled trials and showed that community-based activities – such as music, arts, and sports – produced clinical reductions in depression and anxiety in adults. Buechner et al. say that “social prescribing” – linking individuals to community resources through primary care – is effective but remains underdeveloped in low-income neighborhoods where clinical options are less available. Aleksandra Kupferberg and Greg Hasler deepen this image in their 2023 study, finding that “social cognitive performance is inversely correlated with depression severity” (Kupferberg and Hasler 100631). Simply, the worse a person’s depression becomes, the less ability they have to reach out for help. This creates a self-harming cycle: those who need support are the least neurologically capable of doing it. **The failure to reach out is not a character flaw – it is a symptom of the disorder itself.** Communities do not have time to wait for these struggling

individuals to reach out. When communal infrastructure collapses and individuals lose their capacity to reach out, isolation becomes common, and the crisis Obama mourns accelerates.

Despite modern public awareness of mental health, stigma continues to trap young people before they can reach help; the research of Crockett, Westberg, and Kupferberg together reveals how that trap closes at each level of distress. Marcelo Crockett and colleagues' 2025 meta-analysis, drawing from 97 trials and 44,000 young people from ages ten to twenty-four, finds that stigma – especially self-stigma – is a significant barrier to seeking help, as young people internalize society's negative judgements and continue to view depression as personal failure rather than a treatable condition (Crockett et al. e2454730). Westberg et al.'s 2022 review deepens this image: young people reported fear of social ridicule and pressure to conform to societal norms – particularly young men, who feel vulnerability conflicts with masculine norms – as the primary reason for staying silent about their condition (Westberg et al. 1430). Westberg et al. conclude that psychological safety is not created through awareness campaigns, but through daily reinforcement from families, institutions, and communities that piece together a person's world. But Kepferberg and Hasler reveal the cruelest part: depression itself erodes the social cognitive functions needed to ask for help, meaning that as the illness progresses, the social capacity progresses inversely. Together, these three findings form a self-corroding trap – culture teaches young people that expressing one's suffering is shameful, social environments punish vulnerability, and the illness itself removes the social capacity needed to fight it. When psychological safety disappears, silence becomes the safest option – and silence, as Obama names it in Charleston, is the defining pattern of the national crisis. A third systemic failure is just as impactful as the first two: the unregulated rise of digital

platforms that have replaced face-to-face communal spaces and conversation with algorithms engineered to enhance comparison and conflict. Holly Shannon and colleagues' 2022 meta-analysis in JMIR Mental Health found “moderate but statistically significant correlations between problematic social media use and depression” (Shannon et al. e33450) across their eighteen studies and 9,269 participants, with consistent harms across all ages and genders. **This makes it a population-level phenomenon, not one confined to a vulnerable demographic.** Shannon et al. describe the behavior comparatively to addiction: preoccupation, mood alteration, and withdrawal symptoms. Digital platforms act as highways for social comparison and cyberbullying, which displaces the social interactions that come with face-to-face conversations. **Society has allowed these platforms to absorb the role of traditional civic spaces – community centers, neighborhoods, etc. – without establishing any regulations to protect the people who use them. A generation has grown more digitally connected and isolated simultaneously, and no policies have attempted to address the issue.**

Even for those who overcome stigma and recognize their need for help, a fourth failure blocks the way: a healthcare system that denies access to those who need it most.

Asos Mahmood and colleagues' 2024 study in the Community Mental Health Journal, researching from 2020-2021 NSCH data from 6,655 children, found that 50.8 percent of caregivers reported difficulties obtaining mental healthcare. The barriers include the cost, inadequate insurance coverage, and institutional shortages. Mahmood et al. confirm that childhood rates of anxiety and depression “increased by 29% and 27% respectively between 2016 and 2020” (Mahmood et al. 1534). This data was taken before the COVID-19 pandemic entirely, confirming that the acceleration was structural from the start – not the product of a single event. Untreated childhood depression does not fade; it deepens in adulthood through

despair, substance abuse, and suicidal thoughts. Awareness of the crisis without access to the needed care does not produce help, but rather an educated generation that feels abandoned.

Obama's eulogy makes this clear; it produces abandonment.

Mental illness has been argued to be rooted entirely in genetics and brain chemistry, and

that focusing on the structural causes distracts individuals from medical solutions such as

modern medications and clinical treatments. The fundamentals of human biology are worth

acknowledging: neurological and genetic factors are real and share a space in the crisis.

However, the research doesn't deny biology – it determines whether biologically vulnerable

individuals are protected or further exposed to the same social and structural collapses. Buechner

et al. stated that community-based interventions produced clinically significant results without

altering biology; outcomes changed due to environmental changes. Mahmood et al.'s

documented a 29 percent rise in depression over four years alone, which can not be justified by

genetics alone; the timeline points to environmental causes. Kupferberg and Hasler displayed

that biologically rooted illness, as well as other mental illnesses, worsen when communal support

is not sought, since depression hinders social capacities needed to do so. A second objection may

state that awareness has never been higher, so if the crisis continues to accelerate, individuals

must be refusing help. Crockett et al. and Mahmood et al. answer this directly: awareness is not

the same as access, and short-term awareness campaigns do not evidently produce clinical

long-term change. **Telling a child that help exists while it remains financially or**

geographically out of reach is not treatment. It is a formal version of abandonment.

The mental health crisis in America is not a mystery. It is a predictable outcome of

the four key failures: the collapse of communal infrastructure documented by Buechner et

al. and Kupferberg and Hasler; the cultural stigma enforcing silent conformity confirmed

by Crockett et al., Westberg et al., and Kupferberg and Hasler; the unregulated psychological damage of digital platforms established by Shannon et al.; and a healthcare system that denies care to those who need it most, documented by Mahmood et al. Obama's eulogy **was** a diagnosis of what happens when a society stops tending to its most vulnerable **members** – and a decade of peer-reviewed research proves this diagnosis through credible data and scholarly critical thinking, proving that communities must rebuild their civic infrastructure – the churches, neighborhoods, and social hubs – that once kept individuals in a safety net. Schools and families must replace their cultural stigma surrounding mental illness with daily practices of psychological safety. Lawmakers must close the gaps between the common person and the institutions by providing insurance coverage. And society must demand accountability from the social media platforms that profit from every individual who falls into isolation. Obama stood at the podium in 2015 and named a silence. Ten years later, that silence has a mountain of evidence behind it – millions of unmet appointments, children whose depression went untreated before adulthood, and young men who learned that seeking help meant losing their social image. The research presented here does not offer a second eulogy. It offers a blueprint for reversing the damage created by modern culture. But blueprints only matter if someone picks them up.

Works Cited

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